NURSING WITH LOLO

Medical-Surgical Nursing Overview

A systems-first map of adult assessment, common risks, and priority escalation.

Simple explanations - nursing priorities - memory cues - original practice - rationales

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What you will be able to do

Use these objectives to guide active recall before and after the lesson.

Learning objectives

- Perform a focused cardiovascular assessment and compare findings with baseline.
- Connect left- and right-sided congestion with common assessment findings.
- Recognize findings that require prompt escalation.
- Explain safe monitoring and teaching for a person living with heart failure.
- Complete a focused respiratory assessment in a reliable order.
- Distinguish ventilation from oxygenation.

How to use this guide

- Read the simple overview once.
- Close the guide and explain the priority in your own words.
- Answer the practice items before opening the rationale.
- Return to the lowest-confidence section tomorrow.

The concept, made simple

Plain language first; precise nursing language follows.

Core explanations

- The heart is a pump. Heart failure means it cannot fill or move blood well enough for the body's needs; it does not mean the heart has stopped.
- When blood backs up toward the lungs, breathing can become difficult. When it backs up into the body, swelling and weight gain may appear.
- Poor forward flow can reduce urine output, cool the skin, cause fatigue, and change alertness.
- A single finding matters, but a new cluster or worsening trend is usually more useful than one isolated number.
- Ventilation is air moving in and out; oxygenation is oxygen moving into blood and tissues. A patient can have trouble with one or both.
- The best respiratory assessment combines appearance, rate and pattern, chest movement, sounds, oxygen saturation, and mental status.

Vocabulary move

- Name the body system or safety process.
- Connect the cue to the risk.
- State what the nurse should notice and do first.

Assess before you act

A focused assessment organizes the data that make the next action safe.

What to notice

- Ask about chest discomfort, activity tolerance, orthopnea, nighttime breathlessness, and recent weight change.
- Measure vital signs and oxygen saturation, then interpret them against symptoms and baseline.
- Inspect work of breathing, skin color, alertness, neck veins, and dependent areas for edema.
- Auscultate heart rhythm and lung sounds using a consistent sequence.
- Compare peripheral pulses, temperature, capillary refill, and edema bilaterally.
- Trend intake, output, daily weight, and response to ordered therapies.
- Observe before touching: position, color, alertness, speech, symmetry, and work of breathing.
- Count rate and note depth, rhythm, effort, cough, and secretions.

Trend, do not snapshot

- Compare with baseline.
- Cluster related cues.
- Validate unexpected values.
- Escalate time-sensitive changes.

Priority actions and safety boundaries

The safest answer protects the client while staying inside role, order, and policy.

Priority actions

- For new chest pain, severe dyspnea, syncope, or signs of poor perfusion, stay with the patient and activate the appropriate urgent response.
- Assess airway, breathing, circulation, vital-sign trends, and mental status before routine tasks.
- Compare the current presentation with baseline and report the exact change and time of onset.
- Reassess after every intervention and document the response.
- Escalate a concerning trend even when a single value is not dramatically abnormal.
- For severe distress, stridor, apnea, sudden reduced responsiveness, or rapidly worsening oxygenation, activate the emergency pathway and stay with the patient.
- Assess airway patency and breathing effectiveness before collecting a long history.
- Verify equipment placement and signal while treating the patient, not the device alone.

Safety warnings

- Do not delay escalation for severe dyspnea, chest pressure, cyanosis, new confusion, or signs of shock.
- Do not give a scheduled cardiac medicine without checking required parameters and the current clinical picture.
- Do not assume all crackles or edema are caused by heart failure; assess for other causes.
- Avoid applying one universal oxygen, fluid, or blood-pressure target to every patient.
- Do not delay emergency escalation for stridor, apnea, severe fatigue, cyanosis, or reduced consciousness.
- Do not rely on saturation alone when the patient looks distressed or is becoming drowsy; motion, poor perfusion, device fit, nail products, and skin pigmentation can affect accuracy.
- Oxygen is a medication: use the ordered target, device, and monitoring plan.

Memory that transfers

A memory aid is useful only when it leads back to clinical reasoning.

Memory hooks

- Left backs up to lungs; right backs up to the rest of the body.
- Pump check: rate, rhythm, refill, pulses, pressure, and person.
- Weight is a fluid trend, not just a nutrition number.
- Look, count, listen, compare, trend.
- Oxygen number plus the whole person equals the real assessment.
- Quiet can be critical when effort is high and airflow is low.

Exam reset

- What can harm the client first?
- What data are missing?
- Is the client stable or unstable?
- Which action is least restrictive and within scope?

Clinical judgment in motion

A patient with chronic heart failure becomes breathless while walking to the bathroom and now needs extra pillows. Weight and ankle swelling have increased since yesterday.

Notice

- New exertional dyspnea
- New orthopnea
- Upward weight trend
- Increasing edema

Interpret

- The clustered changes suggest worsening congestion.
- The patient needs an immediate focused breathing and perfusion assessment.

Respond

- Stop exertion, position for comfort, obtain vital signs and oxygen saturation, and assess lungs and perfusion.
- Notify or activate the appropriate response using the facility pathway and report the trend.
- Carry out orders, then reassess and document the response.

Reflect

- Which finding changed first, and how could consistent weight and symptom tracking support earlier recognition?

Practice before the rationale

Choose the best answer using the cue-risk-action sequence.

Question 1

A patient with heart failure has new breathlessness at rest and cannot finish a sentence. What should the nurse do first?

A. Ask about yesterday's diet

B. Stay with the patient and assess airway, breathing, circulation, and vital signs

C. Schedule teaching for later

D. Document the change at the end of the shift

Question 2

Which findings can support worsening fluid congestion in a patient with heart failure? Select all that apply.

A. Increasing orthopnea

B. Consistent downward weight trend

C. New crackles

D. Increasing dependent edema

E. Improved activity tolerance

Answers, rationales, and printable review

Question 1: B

Severe new breathlessness is an urgent change. Immediate ABC assessment and escalation take priority over history, teaching, or delayed documentation.

Exam clue: A sudden breathing change moves ABCs ahead of routine tasks.

Question 2: A, C, D

Orthopnea, crackles, and edema form a congestion pattern. A downward weight trend and improved tolerance do not support worsening congestion by themselves.

Exam clue: Select the findings that all point in the same worsening direction.

Five-point review

- Heart failure produces congestion, reduced forward flow, or both.
- Trend symptoms, lungs, perfusion, weight, intake/output, kidney function, and electrolytes.
- A new cluster of dyspnea, orthopnea, edema, or weight gain deserves prompt review.
- Medication safety depends on assessment and ordered parameters.
- Reassessment closes the loop after every action.

References

MedlinePlus, U.S. National Library of Medicine: Vital signs - https://medlineplus.gov/ency/article/002341.htm
MedlinePlus, U.S. National Library of Medicine: Electrolyte Panel - https://medlineplus.gov/lab-tests/electrolyte-panel/
National Council of State Boards of Nursing: 2026 NCLEX-PN Test Plan - https://www.ncsbn.org/publications/2026-nclex-pn-test-plan
U.S. Food and Drug Administration: Medication Errors Related to CDER-Regulated Drug Products - https://www.fda.gov/drugs/drug-safety-and-availability/medication-errors-related-cder-regulated-drug-products
AHRQ Patient Safety Network: Medication Administration Errors - https://psnet.ahrq.gov/primer/medication-administration-errors